

N-ORS Oral Rehydration Salts (Orange Flavour)

Each 5.2g sachet contains:

Sodium chloride
658mg Potassium chloride
375mg Sodium citrate
735mg Glucose anhydrous
3382mg

Ionic concentration in mmol/liter of: Na^+ 75, K^+ 20, Cl^- 65, Citrate 10 & Glucose 75

Description:

Powder: A white to off white granular powder with odour of orange flavor

Solution: Clear to slight haze solution with odour of orange flavour

Pharmacodynamics:

An ORS is the cotransport of glucose and sodium across the intestinal membrane. The sodium-potassium-ATP pump on the basolateral membrane of the enterocyte provides the gradient that drives the process. Glucose enhances sodium and, secondarily, water absorption. The optimal glucose to sodium ratio to ensure maximal sodium absorption is 1:1.

Pharmacokinetics:

Absorption

Glucose is rapidly absorbed from the small intestine following oral administration. Potassium is well absorbed following oral administration

Distribution

Not known whether glucose, sodium citrate, sodium chloride is distributed into human milk. Potassium enters extracellular fluid and actively transported into cells, intracellular concentration is up to 40 times extracellular concentration.

Intracellular movement augmented by dextrose, insulin and oxygen.

Metabolism

Metabolism of citrates appears to be associated with bicarbonate formation.

Excretion

Glucose is readily metabolized to carbon dioxide and water.

Citrates are excreted in urine, mainly as metabolites; <5% of an oral dose is excreted in the urine unchanged.

Sodium chloride is substantially excreted by the kidneys.

Potassium is excreted principally in urine; small amounts may be excreted via the skin and intestinal tract.

Potassium excretion decreased in patients with renal impairment.

Indication:

For cases of hypotonic dehydration resulting from diarrhea, vomiting and physical exertion.

Recommended dose:

Direction to be used:

Dissolve contents in 250ml of boiled, cool water and to be taken orally. Unused solution should be stored in a refrigerator and discarded 24 hours after preparation.

Infants: One litre over a period of 24 hours

Children: One litre over 8 to 24 hours, according to age

Adults: Drink freely as required, or as directed by the physician

Use only in recommendation dilution

If a child vomits, the caregiver should wait 10 minutes and then begin again to give the drink to the child slowly, small sips at a time.

Drinks should be given from a clean cup. A feeding bottle should not be used. It is difficult to clean

bottles completely, and unclean bottles can contain germs that cause diarrhea.

For adults, 250 to 500ml of oral rehydration solution for every loose motion has been suggested. The dosage for children is 250ml for every loose motion, and for infants is 1 to 1.5 times their usual feed volume. Normal feeding can continue after the initial fluid deficit has been corrected. Breast feeding should continue between administrations of oral rehydration solution.

Route of administration:

Oral administration

Contraindication:

Contraindicated in patients with hyperkalemia, renal failure, anuria or oliguria, severe dehydration with symptoms of shock, severe diarrhoea, glucose malabsorption, inability to drink, severe and sustained vomiting, intestinal obstruction, paralytic ileus, or perforated bowel.

Warning and precautions:

Oral rehydration salts should be reconstituted only with water and at the volume stated. Fresh drinking water is generally appropriate, but freshly boiled and cooled water is preferred when the solution is for infants. The solution should not be boiled

after it is prepared. Other ingredients such as sugar should not be added. Unused solution should be stored in the refrigerator and discarded 24 hours after preparation.

Individuals with heart failure, pulmonary odema, impaired kidney function, hypertension, toxemia of pregnancy or diabetes mellitus should consult their physician before taking this solution.

Oral rehydration solutions are not appropriate for patients with gastro-intestinal obstruction, oliguric or anuric renal failure, or when parenteral rehydration therapy is indicated as in severe dehydration or intractable vomiting.

Continue feeding the child during diarrhea.

Infants under the age of 2 years with severe diarrhoea/vomiting should be seen by a doctor as soon as possible.

If symptoms persist for longer than 24-48 hours, a doctor should be consulted.

Interaction with other medicaments:

There are no known interactions with other medicaments.

Pregnancy and lactation:

There are no known harmful effects when this medicine is used during pregnancy. Mother should continue to breast feed more frequently for infants on mother's milk, as there is more demand for fluids.

Side effects:

- Glucose intolerance may occur in some patients with diarrhoea.
- Vomiting

Overdosage:

In oral electrolyte replacement therapy, toxicity is rare in previously healthy people. In subjects with renal impairment, hypernatraemia and hyperkalaemia might occur.

In the event of significant overdose serum electrolytes should be evaluated by means of full biochemical profile under hospital conditions and the physician should take the appropriate measures. This is particularly important in the very young and in cases of severe hepatic or renal failure.

Effects on Ability to Drive and Use Machine:

Not applicable

Storage:

Store in a dry place below 30°C. Protect from light

Presentation:

Box of 10, 50 or 100 sachets

Manufacturer & Product Registration Holder:

Noripharma Sdn. Bhd. 200701034604 (792633-A)

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